

PROLONGED LABOUR & OBSTRUCTED LABOUR

RECOGNIZE • ASSESS • ACT



**PROLONGED
LABOUR**



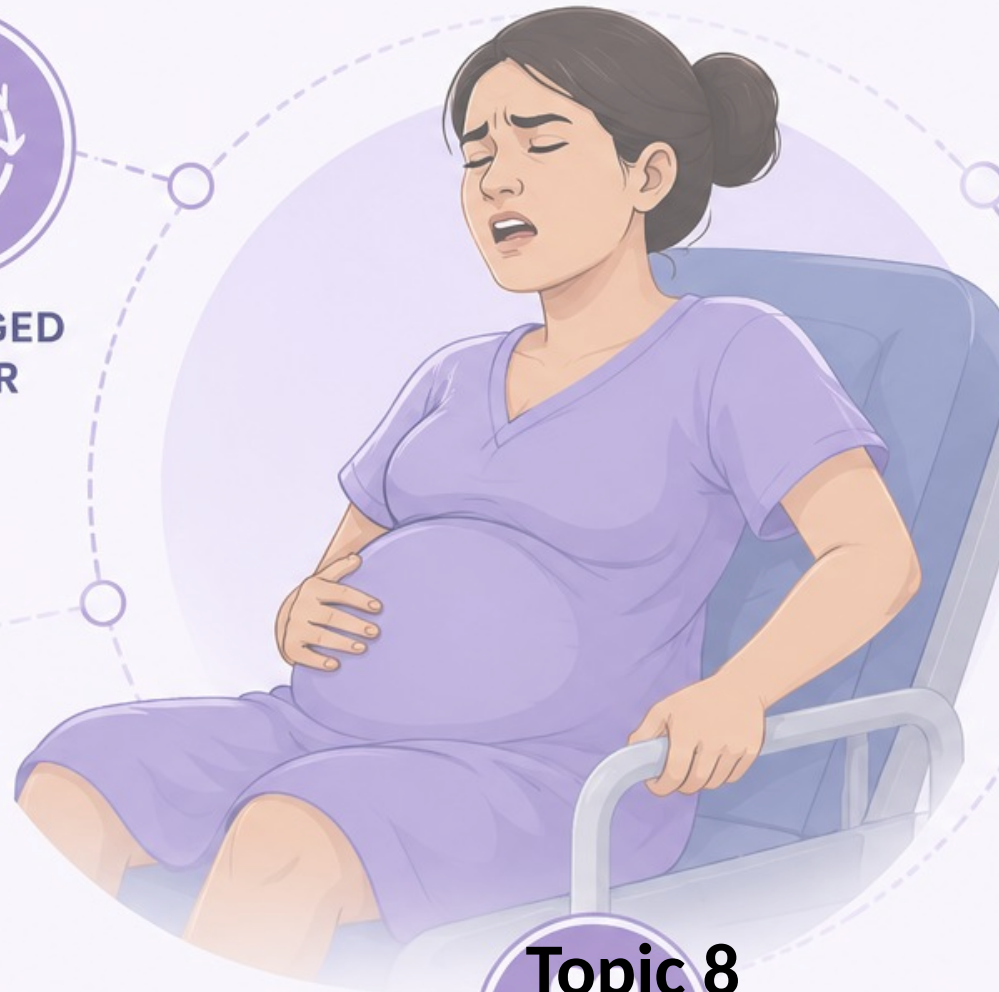
**OBSTRUCTED
LABOUR**



**ASSESS
CAREFULLY**



**PREVENT
COMPLICATIONS**



Topic 8

PARTOGRAPH - CASE STUDY 1

Radha (wife of Gangaram), 26 years of age, third gravida, was admitted at 5:00 am on 10 June 2026 with the complaint of labour pains since 2:00 am. Her membranes had ruptured at 4:00 am. She has two children of the ages of 5 and 2 years. On admission, her cervix was 2 cm dilated.

Plot the following findings on the partograph:

At 09:00 am:

- The cervix is dilated 5 cm.
- She had 2 contractions in 10 minutes, each lasting <20 seconds.
- The FHR is 120 beats per minute.
- The membranes have ruptured and the amniotic fluid is clear.
- Her BP is 120/70 mmHg.
- Her temperature is 36.8°C.
- Her pulse is 80 per minute.

9:30 am: FHR 120, contractions 2/10 each <20 seconds, pulse 80/minute, amniotic fluid clear

10:00 am: FHR 136, contractions 2/10 each <20 seconds, pulse 80/minute, amniotic fluid clear

10:30 am: FHR 140, contractions 2/10 each <20 seconds, pulse 88/minute, amniotic fluid clear

11:00 am: FHR 130, contractions 2/10 each <20 seconds, pulse 88/minute, amniotic fluid clear

11:30 am: FHR 136, contractions 2/10 each <20 seconds, pulse 84/minute, amniotic fluid clear

12:00 noon: FHR 140, contractions 2/10 each <20 seconds, pulse 88/minute, amniotic fluid clear

12:30 pm: FHR 130, contractions 2/10 each <20 seconds, pulse 88/minute, amniotic fluid clear

1:00 pm: FHR 140, contractions 2/10 each <20 seconds, pulse 90/minute, temp. 37°C, BP 100/70 , amniotic fluid clear

At 1:00 pm:

- Cervix dilatation 6 cm
- Amniotic fluid clear and BP 100/70 mmHg

THE SIMPLIFIED PARTOGRAPH

IDENTIFICATION DATA

Name: **Radha**

W/o: **Gangaram**

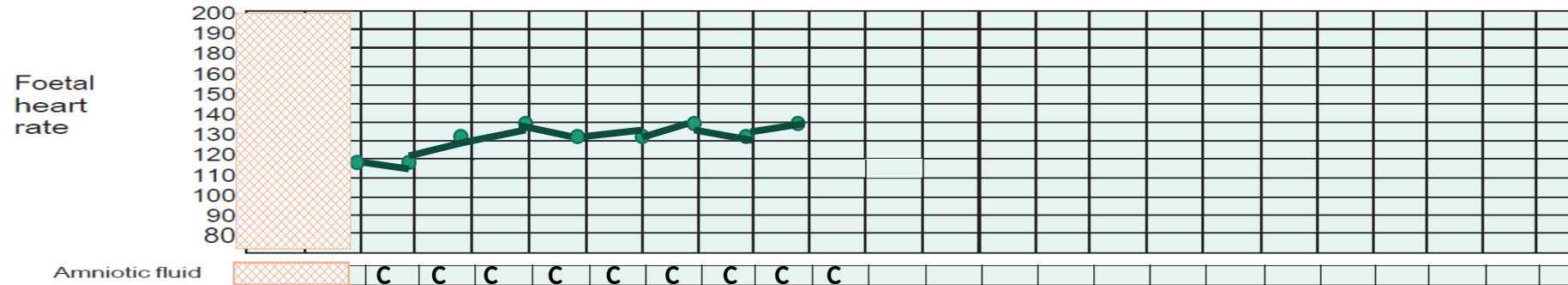
Age: **26 Years**

Parity: **G₃P₂L₂A₀** Reg.No: **XYZ1**

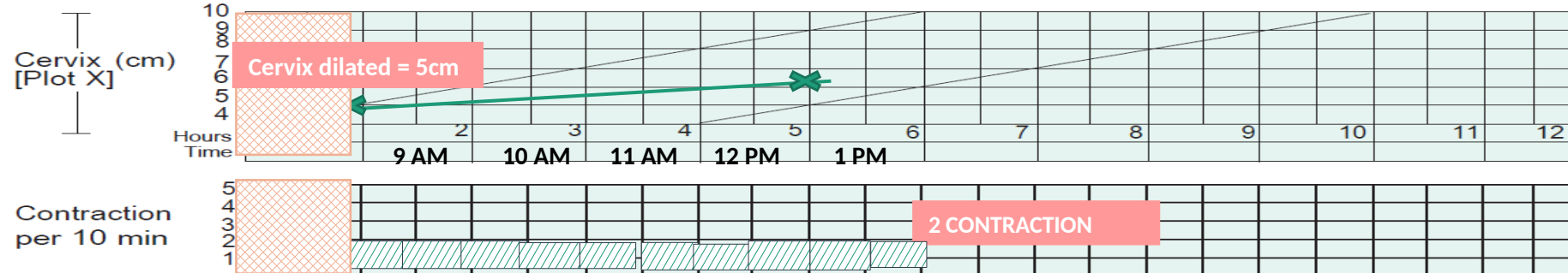
Date & Time of Admission: **11/06/25, 5:00Hrs**

Date & Time of ROM: **11/06/25, 04:00 Hrs**

A) Foetal Condition



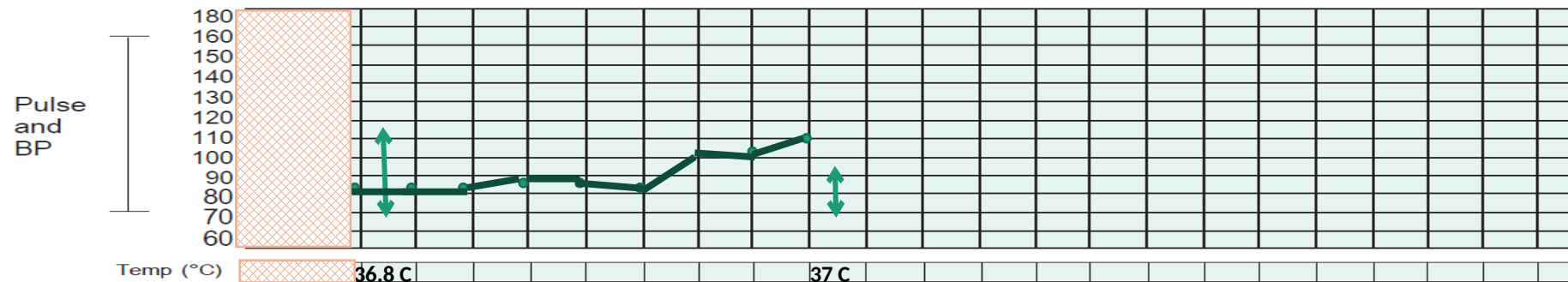
B) Labour



C) Interventions



D) Maternal Condition



PARTOGRAPH - CASE STUDY 2

Radha (wife of Gangaram), 26 years of age, third gravida, was admitted at 5:00 am on 10 June 2026 with the complaint of labour pains since 2:00 am. Her membranes had ruptured at 4:00 am. She has two children of the ages of 5 and 2 years. On admission, her cervix was 2 cm dilated.

Plot the following findings on the partograph:

At 09:00 am:

- The cervix is dilated 5 cm.
- She had 3 contractions in 10 minutes, each lasting 20-40 seconds.
- The FHR is 120 beats per minute.
- The membranes have ruptured and the amniotic fluid is clear.
- Her BP is 120/70 mmHg.
- Her temperature is 36.8°C.
- Her pulse is 80 per minute.

9:30 am: FHR 120, contractions 3/10 each 20-40 seconds, pulse 80/minute, amniotic fluid clear

10:00 am: FHR 136, contractions 3/10 each 20-40 seconds, pulse 80/minute, amniotic fluid clear

10:30 am: FHR 140, contractions 3/10 each 20-40 seconds, pulse 88/minute, amniotic fluid clear

11:00 am: FHR 130, contractions 3/10 each 20-40 seconds, pulse 88/minute, amniotic fluid meconium stained

11:30 am: FHR 130, contractions 3/10 each 20-40 seconds, pulse 84/minute, amniotic fluid meconium stained

12:00 noon: FHR 140, contractions 4/10 each 45 seconds, pulse 100/minute, amniotic fluid meconium stained

12:30 pm: FHR 100, contractions 4/10 each 45 seconds, pulse 100/minute, amniotic fluid meconium stained

1:00 pm: FHR 100, contractions 4/10 each 45 seconds, pulse 110/minute, temp. 37°C, BP 100/70, amniotic fluid meconium stained

At 1:00 pm:

- Cervix dilatation 6 cm
- Amniotic fluid meconium stained and BP 100/70 mmHg

THE SIMPLIFIED PARTOGRAPH

IDENTIFICATION DATA

Name: **Radha**

W/o: **Gangaram**

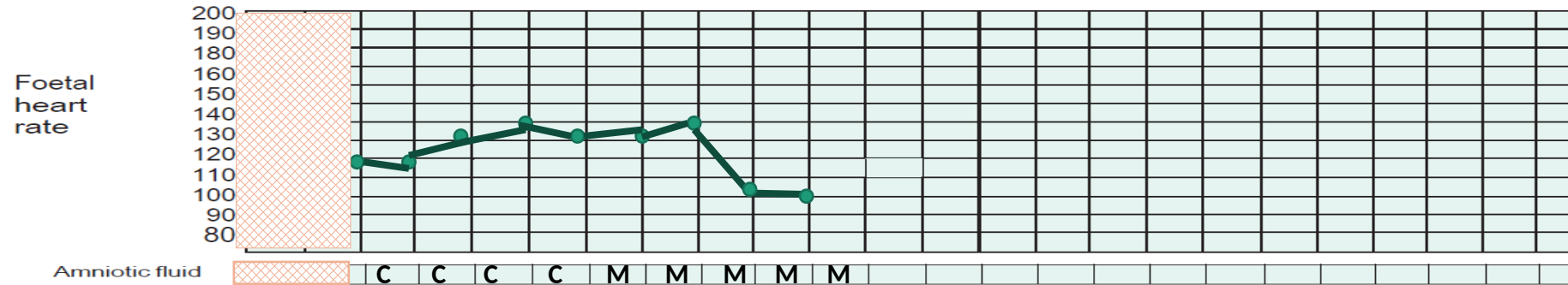
Age: **26 Years**

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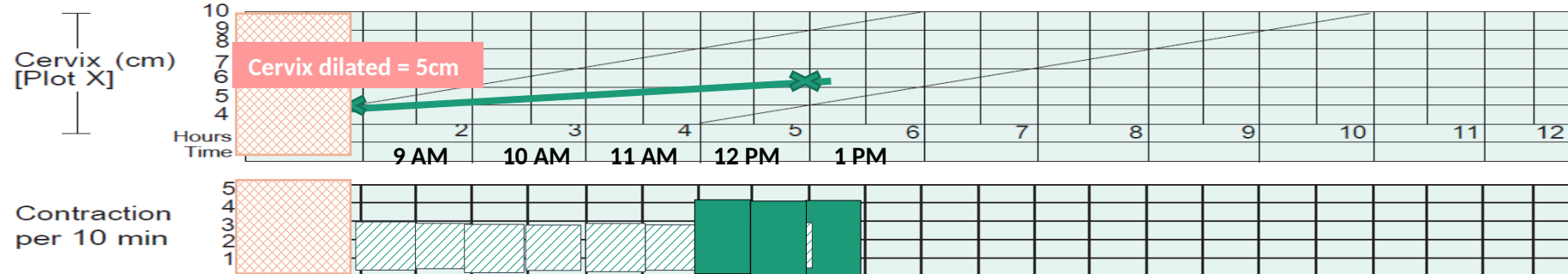
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A) Foetal Condition



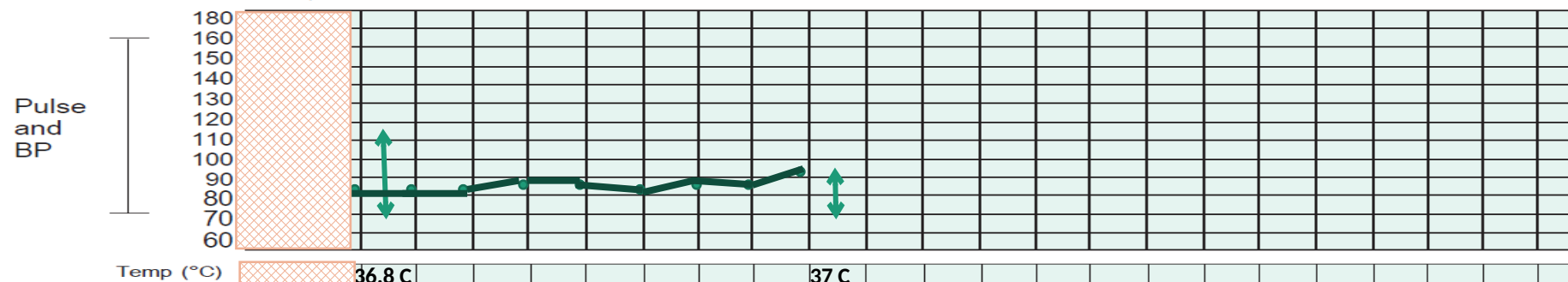
B) Labour



C) Interventions



D) Maternal Condition



PROLONGED LABOUR & OBSTRUCTED LABOUR

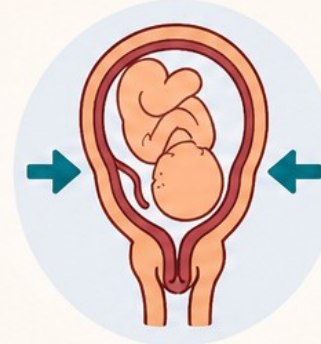
RECOGNIZE • ASSESS • ACT

DEFINITIONS



PROLONGED LABOUR

After onset of regular, rhythmical painful contractions accompanied by cervical dilatation i.e. true labour where labour is longer than **24 hours**.



OBSTRUCTED LABOUR

Means in spite of strong uterine contractions, the foetus cannot descend because of mechanical factors due to fetal or maternal reasons; at the inlet, within cavity or outlet.

FACTORS INFLUENCING DELIVERY PROCESS – PPP



POWER

(Uterine contraction strength)



Weak uterine contractions lead to unsatisfactory progress of labour or prolonged labour.



PASSAGE

(Pelvic Adequacy)



Inadequate pelvis in relation to presenting part leads to CPD and thus obstructed labour.



PASSENGER

(Fetal condition, lie, and presentation)



The Malpositions or malpresentations or any congenital anomaly can lead to prolonged or obstructed labour.



Malpositions: are abnormal position of the vertex of the foetal head (with occiput as the reference point) relative to the maternal pelvis.

Malpresentations: of the fetus are all presentations other than vertex.

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COMPLICATIONS



MATERNAL COMPLICATIONS



Maternal death



Uterine rupture



Fistulae



Premature rupture of membranes



Slow dilatation or edema of cervix



Maternal fatigue



Sepsis



FOETAL COMPLICATIONS



Foetal death



Birth asphyxia and its complications



Caput succedaneum



Excessive moulding of fetal skull



Sepsis



**TIMELY RECOGNITION.
SAFE MOTHER. SAFE BABY.**



Early recognition, prompt assessment and appropriate action can prevent complications and save lives.

The Labor Timeline Dashboard: Diagnostic Thresholds

Prolonged Latent Phase



Diagnostic Alert: True labor, but cervical dilation remains < 4 cm after 8 hours.

Prolonged Active Phase



Diagnostic Alert: Dilation reaches ≥ 5 cm but progresses slower than expected.

Time Limits: Lasting > 12 hours (Nulliparous) or > 10 hours (Multiparous).

Prolonged Second Stage (Expulsive)

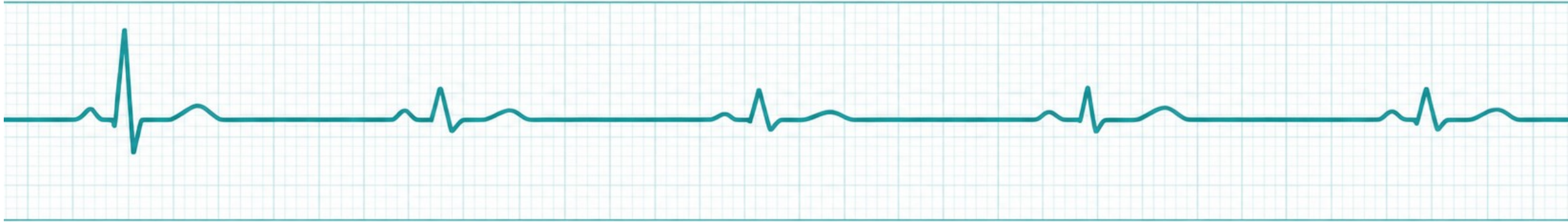


Diagnostic Alert: Failure to deliver within specific pushing windows after full dilation.

Time Limits (Active Pushing): > 2 hours (Nulliparous) or > 1 hour (Multiparous).

Diagnosing Weak Power: Inadequate Uterine Activity

This condition can occur at any time during pregnancy.



1 Frequency

< 3

contractions occurring within
a 10-minute window

2 Duration

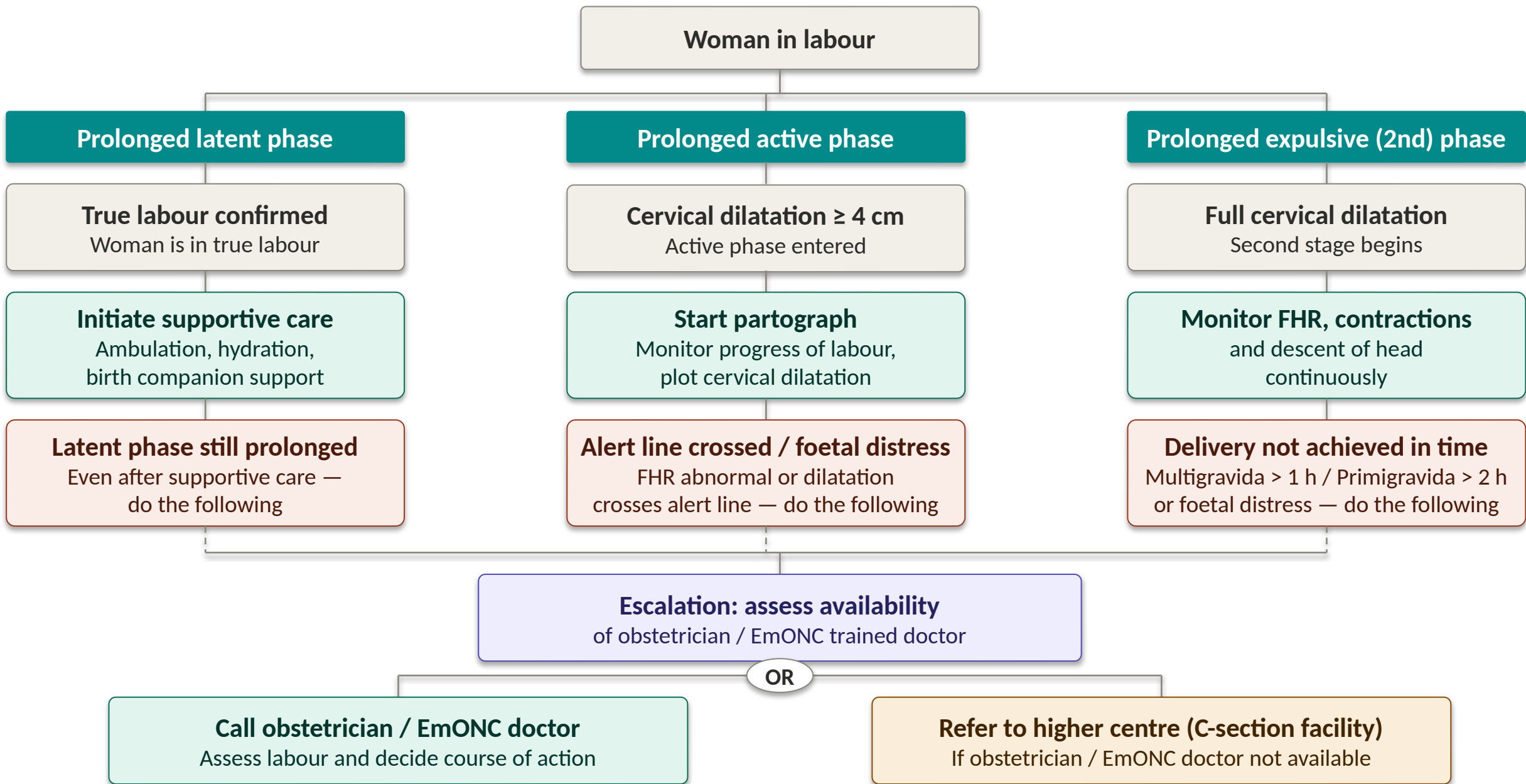
< 40s

maximum duration of each
individual contraction

3 Anytime During Pregnancy



These leads to
Prolonged Labor



■ Phase header ■ Action / monitoring ■ Escalation trigger ■ Decision point ■ Refer if unavailable

OBSTRUCTED LABOUR

WHAT IS OBSTRUCTED LABOUR?



- Obstructed labour is a clinical diagnosis that can occur at any time during active phase or second stage of labour. Obstructed labour means that uterine activity is good but there is still no descent of fetus.



- It can occur due to CPD, malpresentations/malposition, foetal abnormalities.



- Obstructed labour can be prevented by proper obstetric examination and monitoring of progress of labour.

ANALOGY

Good effort but no progress due to obstruction.



Just like a car hitting a wall,
strong contractions (effort) cannot result
in descent (progress) when there is an obstruction.

Clinical Picture

- History, age, parity
- Stature of women
- Previous operative delivery
- Previous still birth
- Duration of labour so far
- Previous history of babies with developmental anomalies
- Progress of labour
- Rupture of membranes and their timing

Findings of Clinical Examination

General examination

- Physical and mental exhaustion
- Dehydration
- ketoacidosis
- Fever
- Hemorrhagic urine in catheter
- Shock due to ruptured uterus or sepsis

Foetal condition: try and listen for FHR

Vaginal examination:

- Foul smelling meconium may be seen
- Amniotic fluid may have already drained away
- Edema of the vulva
- Vagina hot and dry
- Large caput

Management: Obstructed Labour



- In ideal situation, complications from obstructed labour can be avoided through timely identification of cephalo-pelvic disproportion and appropriate monitoring of progress of labour
- If obstructed labour has been confirmed, do the following:



1 Rehydrate the patient

- Start an i/v line R/L or normal saline @ 25-30 drops/min



2 Give antibiotics

- Inj. Ampicillin 1 gm IV after sensitivity testing
- Inj. Gentamicin 80 mg iv
- Inj. Metronidazole 500 mg iv



3 Refer the patient to FRU

Core Clinical Directives for Maternal Safety



Rule 1: Diagnostic Vigilance

Prolonged and obstructed labor must be diagnosed early and managed promptly to prevent severe maternal and fetal outcomes. It is a leading cause of maternal death.



Rule 2: The Partograph is Non-Negotiable

The partograph is the primary tool to identify early deviations from normal fetal and maternal conditions. Use it universally to monitor the progress of labor.



Rule 3: Strict Limits on Augmentation

Labor augmentation should only be performed in clearly indicated cases, and strictly at fully equipped centers under expert medical guidance. Never attempt unguided augmentation during mechanical obstruction.

A SINCERE PLOT IN PARTOGRAPH CAN PREVENT TWO FLATLINE IN MONITOR



A partograph is more than a chart—it's a life-saving tool. A sincere plot helps us identify danger early, take timely action, and prevent tragedy.

**Your careful plot today
can bring two lives tomorrow.**



PLOT SINCERELY



DETECT EARLY



SAVE TWO LIVES



SINCERE PLOT. TIMELY ACTION. TWO LIVES SAVED.